

Respiratory

R-07 CHF / Pulmonary Edema BLS

Revised 3/1/2000

PRIORITIES:

- ABCs
- Determine degree of physiologic distress:
 - Respiratory rate >20, use of accessory muscles, cyanosis, inadequate ventilation, depressed level of consciousness
- Maintain airway, provide oxygen and ventilatory support;
- Determine which causes best fit patient signs and symptoms, initiate treatment;
- Assure an advanced life support response;

Acute Pulmonary Edema

Acute onset of respiratory difficulty; may have history of cardiac, rales, occasional wheezes.

1. Ensure a patent airway (suction as necessary).
2. Be prepared to support ventilation with appropriate airway adjuncts;
3. OXYGEN THERAPY – Begin oxygen at 6 liters/minute by nasal cannula or 10 liters/minute by mask. If there is a history of Chronic Obstructive Pulmonary Disease (COPD), observe for respiratory depression and support respiration as needed. DO NOT withhold oxygen from a patient in cardiorespiratory distress because of a history of COPD.
4. Place patient in position of comfort if conscious. If depressed level of consciousness, position on left side;
5. Assist advanced life support personnel with patient packaging and movement to ambulance after the unit arrives.
6. Consider;
 - Assist patient with his/her medications if available;
 - Limit any physical exertion or movement the patient may be attempting;
 - Attempt to reduce patient anxiety;
 - Positive pressure ventilation if patient is semi- or unconscious.

Respiratory

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PRIORITIES

- ABCs
- Determine degree of physiologic distress
- Evaluate respiratory rate, use of accessory muscles, cyanosis, ventilatory effort, level of consciousness
- Maintain airway, provide oxygen and ventilatory support PRN
- Early Transport, in a sitting position if possible
- Anticipate need for possible intubation
- Early contact of Receiving Hospital

Stabilize Airway

Oxygen therapy based on patient's condition. Assist ventilations if necessary.

Consider CPAP at 10cm H2O

Notify receiving hospital early so hospital CPAP can be ready on arrival.

Cardiac Monitor

Consider 12 Lead EKG

IV/IO NS TKO OR NS Lock

NITROGLYCERIN

0.4 mg SL if systolic BP >100.

May repeat q 5 minutes until condition improves or SBP is <100

Max dose of up to 6 doses

CONTACT BASE PHYSICIAN FOR REPEAT ORDERS.

DISRUPTED COMMUNICATIONS

In the event of a "disrupted communications" situation, Solano County Paramedics may utilize all portions of this treatment protocol without Base Hospital Contact as needed to stabilize an immediate patient.